Export Malaysia

18 cm

20 cm

H-PERAN

LF-0146

Composition : Each 2 mL ampoule contains :

Metoclopramide HCl eq to Anhydrous Metoclopramide HCI	10 mg
Sodium Metabisulfite	4 mg
Benzyl Alcohol	21 mg

Indication :

Adult population

H-PERAN is indicated for use in adults for: Prevention of post-operative nausea and vomiting, symptomatic treatment of nausea and vomiting, including nausea and vomiting induced by migraine attacks, prevention of radiotherapy-induced nausea and vomiting.

Pediatric population

H-PERAN is indicated in children aged 2 to 18 years for: Prevention of delayed chemotherapy-induced nausea and vomiting as a second-line option, prevention of post-operative nausea and vomiting as a second-line option.

Dose and Administration :

The solution can be administered by the intravenous or intramuscular route.
The intravenous doses must be administered as a slow bolus (for at least 3 minutes).

All indication (adults)

A single 10 mg dose is recommended for the prevention of post-operative nausea and vomiting.
The recommended dose for the symptomatic treatment of nausea and vomiting, including nausea and vomiting induced by migraine attacks and for the prevention of radiotherapy-induced nausea and vomiting is 10 mg per dose, 1 to 3 times daily.
The maximum recommended daily dose is 30 mg or 0.5 mg/kg.
Treatment duration when administering by injection should be as short as possible and a switch to administration via oral or rectal route should be instituted as quickly as possible.

All indications (children from 2 to 18 years of age)

The recommended dosage is 0.1 to 0.15 mg/kg, 1 to 3 times daily, by intravenous route.
The maximum daily dose is 0.5 mg/kg.
Dosing table

Age	Body weight	Dose	Frequency
2-3 years	10-14 kg	1 mg	Up to 3 times daily
3-5 years	15-19 kg	2 mg	Up to 3 times daily
5-9 years	20-29 kg	2.5 mg	Up to 3 times daily
9-18 years	30-60 kg	5 mg	Up to 3 times daily
15-18 years	Over 60 kg	10 mg	Up to 3 times daily

For the prevention of delayed chemotherapy-induced nausea and vomiting, the maximum treatment duration is 5 days.
For the prevention of post-operative nausea and vomiting, the maximum treatment duration is 48 hours.
NOTE: Total daily dose of metoclopramide, especially for children and young adults, should not normally exceed 0.5 mg/kg of body weight.

Frequency of administrations :

A minimum interval of 6 hours between two administrations is to be respected, even if vomiting or rejection of the dose occurs.

Special populations

Elderly subjects
In elderly subjects, a dose reduction should be considered, based on kidney and liver function, and overall frailty.

Kidney failure
In patients with end-stage kidney failure (creatinine clearance ≤ 15 ml/min), the daily dose should be reduced by 75 %.
In patients with moderate to severe kidney failure (creatinine clearance between 15 and 60 ml/min), the dose should be reduced by 50%.

Liver failure
In patients with severe liver failure, the dose should be reduced by 50%.
Other pharmaceutical forms may be more suitable for these patient populations.

Pediatric Population
Metoclopramide is contraindicated in children aged less than two years.

Pharmacology / Pharmacokinetics :

Absorption : Metoclopramide is rapidly and well absorbed. Onset of action is 1 to 3 minutes following an IV dose, 10-15 minutes following IM administration.
Blood concentration : Peak plasma concentrations occur at about 1-2 hrs.
Half-life : Elimination half-life in individuals with normal renal function is 5-6 hrs.
Distribution : The whole body volume of distribution is high (about 3.5 L/kg) which suggests extensive distribution of the drug to the tissues.
Metabolism : The drug is not extensively bound to plasma proteins (about 30%).
Excretion : Approximately 85% of an orally administered dose appears in the urine within 72 hours. Of the 85% eliminated in the urine about one-half is present as free or conjugated metoclopramide.

Pharmacodynamics :

Metoclopramide stimulates motility of the upper gastrointestinal tract without stimulating gastric, biliary, or pancreatic secretions. Its mode of action is unclear. It seems to sensitize tissues to the action of acetylcholine. The effect of Metoclopramide on motility is not dependent on intact vagal innervation, but it can be abolished by anticholinergic drugs. Metoclopramide increases the tone and amplitude of gastric (especially antral) contractions relaxes the pyloric sphincter and the duodenum and jejunum resulting in accelerated gastric emptying and intestinal transit. It increases the resting tone of the lower esophageal sphincter.

The antiemetic properties of Metoclopramide appear to be a result of its antagonism of central and peripheral dopamine receptors.
Dopamine produces nausea and vomiting by stimulation of the medullary chemoreceptor trigger zone (CTZ), and Metoclopramide blocks stimulation of the CTZ by agents like 1-dopa or apomorphine which are known to increase dopamine levels or to possess dopamine-like effects.
Metoclopramide abolishes the slowing of gastric emptying caused by apomorphine.

Side effects / Adverse reactions :

The most frequent adverse reactions to metoclopramide are restlessness, drowsiness, fatigue and lassitude, which occur in approximately 10% of patients. Less frequently, insomnia, headache, dizziness, nausea, galactorrhea, gynecomastia, or bowel disturbances may occur. Urinary frequency and incontinence also have been reported. Rarely, hepatotoxicity, manifested as jaundice and alterations in liver function test results, has occurred in patients receiving metoclopramide concomitantly with other drugs with hepatotoxic potential.

Metoclopramide is a potent stimulator of prolactin secretion in both genders; however, the clinical importance of the drug's effect on prolactin has not been fully determined. Impotence secondary to hyperprolactinemia can occur. Serum prolactin concentration usually returns to normal within 1 week following discontinuance of metoclopramide; adverse effects associated with increased serum prolactin concentration usually subside within a few weeks to months following discontinuance of the drug.
Extrapyramidal Reactions (EPS) : Acute dystonic reactions, the most common type of EPS associated with Metoclopramide, occur in approximately 0.2% of patients (1 in 500) treated with 30 to 40 mg of Metoclopramide per day. In cancer chemotherapy patients receiving 1-2 mg/kg per dose, the incidence is 2% in patients over the ages of 30-35, and 25% or higher in children and young adults who have not had prophylactic administration of diphenhydramine. Symptoms include involuntary movements of limbs, facial grimacing, torticollis, oculogyric crisis, rhythmic protrusion of tongue, bulbar type of speech, trismus, opisthotonus (tetanus-like reactions) and rarely, stridor and dyspnea, possibly due to laryngospasm; ordinarily these symptoms are readily reversed by diphenhydramine.

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Size : 18 x 20 cm (WxH) Colours : K=100

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18 cm

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Parkinsonian-like symptoms may include bradykinesia, tremor, cogwheel rigidity, mask-like facies. Tardive dyskinesia most frequently is characterized by involuntary movements of the tongue, face, mouth or jaw, and sometimes by involuntary movements of the trunk and/ or extremities; movements may be choreoathetotic in appearance.

Drug interactions :

Anticholinergics, Narcotic analgesics: The effects of metoclopramide on GI motility are antagonized by these agents.
Alcohol, sedatives, hypnotics, narcotic or tranquilizers : More sedative effects can occur when Metoclopramide is given with these agents.
MAO inhibitors : Since Metoclopramide releases catecholamines in patients with essential hypertension, use cautiously, if at all, in patients receiving MAO inhibitors.
Digoxin : Absorption of digitoxin from the stomach may be diminished.
Lithium : The rate and extent absorption of lithium may be enhanced.
Dopaminergics : Metoclopramide is a dopamine receptor antagonist.

Warning

This preparation contains Sodium metabisulfite that may cause serious allergic type reactions in certain susceptible patients. Do not use if known to be hypersensitive to bisulfite.

Special Warnings and precautions For Use :

- As this preparation contains benzyl alcohol, its use should be avoided in children under two years of age. Not to be used in neonates.
- Avoid doses exceeding 0.5mg/kg/day.
- Extrapyramidal effects, especially dystonic reaction of metoclopramide are more likely to occur in children shortly after initiation of therapy, and usually with doses higher than 0.5 mg per kg of body weight per day.

Neurological disorders

Extrapyramidal disorders may occur, particularly in children and young adults, and/or when high doses are used. These reactions generally occur at the beginning of treatment, and can occur after a single dose. If extrapyramidal symptoms occur, metoclopramide should be discontinued immediately. These effects are generally completely reversible after treatment discontinuation; however, symptomatic treatment may be required (benzodiazepines in children, and/or anticholinergic antiparkinsonian medicinal products in adults). An interval of at least six hours should be respected between each dose even if vomiting or rejection of the dose occurs, in order to avoid overdose. Long-term treatment with metoclopramide may cause potentially irreversible tardive dyskinesia, particularly in elderly subjects. Treatment should not exceed 3 months because of the risk of tardive dyskinesia. Treatment must be discontinued if clinical signs of tardive dyskinesia occur. Neuroleptic malignant syndrome has been described with metoclopramide in combination with neuroleptics and with metoclopramide monotherapy.
Metoclopramide must be immediately discontinued if symptoms of neuroleptic malignant syndrome develop, and appropriate treatment should be initiated. Particular caution should be exercised in patients with underlying neurological disorders, and in patients receiving other centrally-acting drugs. Symptoms of Parkinson's disease may also be exacerbated by metoclopramide.

Methemoglobinemia

Methemoglobinemia, which could be related to NADH-cytochrome b5 reductase deficiency, has been reported. If this occurs, treatment must be immediately and permanently discontinued, and appropriate measures initiated (such as treatment with methylene blue).

Cardiac disorders

Serious cardiovascular undesirable effects, including cases of severe bradycardia, circulatory collapse, cardiac arrest and QT prolongation have been reported during administration of metoclopramide by injection, particularly via the intravenous route. Particular caution should be exercised when administering metoclopramide, particularly via the intravenous route, in elderly subjects, patients with cardiac conduction disorders (including QT prolongation), patients with electrolyte imbalance, bradycardia, and patients taking other drugs known to prolong QT interval.
The intravenous injection must be given as a slow bolus (of at least 3 minutes'duration) in order to reduce the risk of undesirable effects (e.g.hypotension, akathisia).

Kidney or liver failure

In patients with kidney failure or severe liver failure, a dose reduction is recommended.

Use in Pregnancy & Lactation :

Reproduction studies performed in rats, mice and rabbits by IV, IM, SC and oral routes at maximum levels ranging from 12-250 times the human dose have demonstrated no impairment of fertility or significant harm to the fetus due to metoclopramide. There are, however, no adequate and well-controlled studies in pregnant women. Because animal reproductive studies are not always predictive of human response, H-PERAN should be used during pregnancy only if clearly needed. It is not known whether H-PERAN is excreted in human milk. Because many drugs are excreted in human milk, caution should be exercised when metoclopramide is administered to a nursing mother.

Contraindication :

- Hypersensitivity to the active substance or to any of the excipients listed
- Gastrointestinal haemorrhage, mechanical obstruction or gastro-intestinal perforation for which the stimulation of gastrointestinal motility constitutes a risk.
- Confirmed or suspected pheochromocytoma, due to the risk of severe hypertension episodes.
- History of neuroleptic or metoclopramide-induced tardive dyskinesia
- Epilepsy (increased crises frequency and intensity)
- Parkinson's disease
- Combination with levodopa or dopaminergic agonists.
- Known history of methaemoglobinaemia with metoclopramide or of NADH cytochrome-b5 deficiency
- Use in children less than 2 year of age

Symptoms and Treatment for Overdosage and Antidote(s) :

Symptoms of overdosage may includ drowsiness, disorientation and extrapyramidal reaction. Anticholinergic or antiparkinson drugs and antihistamines with anticholinergic properties may be helpful in controlling the extrapyramidal reactions. Symptoms are self-limiting and usually disappear within 24 hrs. Treatment of overdosage generally involves symptomatic and supportive care.

Storage conditions : Store below 30°C. Protect from light.

Shelf-Life : 4 years

Product description : Clear colorless sterile solution in an amber glass ampoule - 2 mL.

Pack size presentation : 1 x [50 x 2 mL/amp.]

Registration numbers :

Thai Reg. No. 1A 1078/30
Mal. Reg. No. MAL20012697AZ

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